

SENATE No. 842

The Commonwealth of Massachusetts

PRESENTED BY:

Joanne M. Comerford

To the Honorable Senate and House of Representatives of the Commonwealth of Massachusetts in General Court assembled:

The undersigned legislators and/or citizens respectfully petition for the adoption of the accompanying bill:

An Act to address medical debt through hospital financial assistance reform.

PETITION OF:

NAME:	DISTRICT/ADDRESS:	
<i>Joanne M. Comerford</i>	<i>Hampshire, Franklin and Worcester</i>	
<i>Steven Owens</i>	<i>29th Middlesex</i>	<i>1/24/2025</i>
<i>James B. Eldridge</i>	<i>Middlesex and Worcester</i>	<i>3/19/2025</i>

SENATE No. 842

By Ms. Comerford, a petition (accompanied by bill, Senate, No. 842) of Joanne M. Comerford and Steven Owens for legislation to offer financial assistance for medical debt to those below the federal poverty level. Health Care Financing.

The Commonwealth of Massachusetts

In the One Hundred and Ninety-Fourth General Court
(2025-2026)

An Act to address medical debt through hospital financial assistance reform.

Be it enacted by the Senate and House of Representatives in General Court assembled, and by the authority of the same, as follows:

1 SECTION 1. Chapter 118E of the General Laws, as appearing in the 2022 Official
2 Edition, is hereby amended by inserting after section 69A the following section:-

3 Section 69B. (a) As used in this section, the following words shall have the following
4 meanings unless the context clearly requires otherwise:

5 (i) “Bad debt”, as defined in section 64 of chapter 118E.

6 (ii) "Charity care", free or reduced-cost health care services provided by hospitals to
7 eligible persons in accordance with this section.

8 (iii) “Debt buyer”, an individual or entity that is engaged in the business of purchasing
9 medical debt for collection purposes, whether it collects the debt itself or hires a third party for
10 collection or an attorney for litigation in order to collect such debt. Debt buyers purchasing
11 medical debt for purposes of forgiving patient debt are excluded from this definition.

(iv) “Eligible person”, any low- or moderate-income person who meets the criteria set forth in the uniform application for a financial assistance policy and the household income eligibility as set forth in subsection (b) of this section.

(v) "Financial assistance policy", a policy administered by a hospital describing how the hospital will provide financial assistance, which includes free or reduced-cost health care services, to persons who meet the criteria for financial assistance as determined in this section.

(vi) “Hospital”, an acute hospital as defined in section 64 of chapter 118 and including any facilities that provide outpatient health services and are operated under the common control or ownership of a hospital.

(vii) “Medical hardship”, a person whose household income is not more than 600 per cent of the federal poverty level whose out-of-pocket medical costs in the past 12 months accumulated to more than 10 per cent of the person’s gross household income.

(viii) “Medically necessary services”, health care services including diagnostic testing, preventive services and after care that are appropriate to the patient’s diagnosis or condition.

(b) Hospitals shall develop, implement and maintain a financial assistance policy for free or discounted care for emergency and medically necessary services available to all eligible persons at any point after such service is rendered, including during the collections process, pursuant to the following:

(i) All eligible persons whose household income is not more than 200 per cent of the federal poverty level, as calculated pursuant to the regulations of the executive office, or who are

experiencing medical hardship as defined in this section, shall be entitled to free care for the full amount of the patient responsibility portion of their hospital bill;

(ii) All eligible persons whose household income is between 201 and 300 per cent of the federal poverty level, as calculated pursuant to the regulations of the executive office, shall be entitled to a 75 per cent discount for the full amount of the patient responsibility portion of their hospital bill;

(iii) All eligible persons whose household income is between 301 and 350 per cent of the federal poverty level, as calculated pursuant to the regulations of the executive office, shall be entitled to a 50 per cent discount for the full amount of the patient responsibility portion of their hospital bill; and

(iv) All eligible persons whose household income is between 351 and 400 per cent of the federal poverty level, as calculated pursuant to the regulations of the executive office, shall be entitled to a 25 per cent discount for the full amount of the patient responsibility portion of their hospital bill.

(c) When determining eligibility for a financial assistance policy, hospitals shall not consider the following:

(i) the eligible person's immigration status;

(ii) the total cost of services provided; or

(iii) contract language between a provider and payor that would otherwise limit a hospital's ability to offer free or discounted care to an eligible person under this section. Any such contract language shall be considered void.

(d) Written and verbal notification of a hospital's financial assistance policy shall be provided during the intake and registration process as well as when a person is discharged from the hospital.

(e) Each hospital shall post and prominently display notice of their financial assistance policy in any language that is spoken by the lesser of 1,000 individuals or 5 per cent of the community served. Notice must be displayed in at least the following locations:

(i) areas where patients are admitted or registered;

(ii) emergency departments, if applicable; and

(iii) financial service or billing areas that are accessible to patients.

(f) Each hospital shall make current versions of the financial assistance policy, a plain language summary of the policy and the uniform application form, as determined by subsection (i), available on the hospital's website, online patient portals and electronic or paper billing statements in any language that is spoken by the lesser of 1,000 individuals or 5 per cent of the community served.

(g) Billing statements and other written communications concerning billing or collection of a hospital bill must include the following or a substantially similar statement prominently displayed on the first page of the billing statement in both English and the second most spoken language in the hospital's service area:

You may qualify for free care or a discount on your hospital bill, even if you have insurance. Please contact our financial assistance office at [website] and [phone number].

(h) A hospital shall issue notice upon submission of a completed financial assistance policy application that the applicant may disregard any bills until the hospital has rendered a decision on the application. A hospital shall also issue notice of any denial and or approval of an application for the financial assistance policy with information on how to appeal the denial and shall establish an appeals process under which it will evaluate the denial of an application.

(i) The executive office and the health policy commission, in consultation with the health safety net office, the office of the attorney general, Massachusetts Health and Hospital Association, Inc., Health Care For All, Inc., a community organization representing low-income individuals, a legal services organization representing low-income individuals and any other representative as determined necessary by the executive office of health and human services, shall develop the following:

(i) a uniform financial assistance policy application to be used by each hospital required under this section to offer a financial assistance policy;

(ii) guidelines for training relevant staff on the uniform financial assistance policy application and financial policies and the use of interpreter services for the purpose of financial counseling. Relevant staff should include anyone who performs functions relating to registration, admissions or billing whether employed by the hospital or as a part of a third-party contract to perform these functions; and

(iii) standards for presumptive eligibility for financial assistance policies based on a person's eligibility for other state programs.

(j) Hospitals must report annually to the center for health information and analysis the following aggregated data broken down by age, gender, race, ethnicity, primary language spoken and insurance status, as practicable:

(i) the number of people that have applied for a financial assistance policy;

(ii) the number of financial assistance policy applications denied or approved, or deemed incomplete;

(iii) financial reporting data including aggregate charity care, bad debt and services eligible for or reimbursed by the health safety net; and

(iv) the amount of bad debt sold to a debt buyer for collections purposes; and

(v) any other relevant data deemed necessary.

(k) The health policy commission, pursuant to data collected under subsection (j) shall provide an annual report to the joint committee on health care financing and the senate and house committees on ways and means on the use of financial assistance policies and the impact on patients and hospitals. Annual reports shall be posted publicly on the health policy commission's website.

SECTION 2. The requirements in subsection (b) of section 1 shall take effect 1 year after the effective date of this act.

SECTION 3. The requirements in subsection (i) of section 1 shall be completed not later than 1 year after the effective date of this act.